

Richmond Agitation-Sedation Scale (RASS)

adults (ICU/ward); Paeds STG also lists RASS or COMFORT-B · casualty / ward / high care

RASS -5 to +4

Score	Term	Description +	Stimulus
+4	Combative	Overtly combative, violent, immediate danger to staff	Without stimulation
+3	Very agitated	Pulls or removes tube(s) or catheter(s); aggressive	
+2	Agitated	Frequent non-purposeful movement, fights ventilator	
+1	Restless	Anxious but movements not aggressive or vigorous	
0	Alert and calm (target)		Verbal stimulation
-1	Drowsy (target)	Not fully alert, but sustained awakening (eye opening/eye contact) to voice (≥ 10 s)	
-2	Light sedation (target)	Briefly awakens with eye contact to voice (<10 s)	
-3	Moderate sedation	Movement or eye opening to voice (but no eye contact)	Physical stimulation
-4	Deep sedation	No response to voice, but movement or eye opening to physical stimulation	
-5	Unarousable	No response to voice or physical stimulation	

HOW TO SCORE

- Observe the patient**
Alert and calm $\rightarrow 0$ · restless or agitated $\rightarrow +1$ to $+4$
- If not alert: say name loudly, ask to open eyes and look at you**
Eye contact sustained (≥ 10 s) $\rightarrow -1$ · eye contact <10 s $\rightarrow -2$ · any movement, no eye contact $\rightarrow -3$
- No response to voice: shake shoulder, then sternal rub if no response**
Any movement $\rightarrow -4$ · no response $\rightarrow -5$

TARGETS & SA STG

Target RASS -2 to 0; lighter sedation preferred over deeper +

Exclude and manage delirium before routine sedation +

Analgesia first (analgo-sedation), then sedate if still needed +

Sedation indicated for: severe acute respiratory failure, status epilepticus, raised ICP/TBI, status asthmaticus +

CAM-ICU-7: RASS 0 = 0 · RASS $\pm 1 = 1$ · RASS $>+1$ or $<-1 = 2$ (altered consciousness) +

Children: RASS or COMFORT-B, 4-8-hourly +

COMMONLY USED SEDATIVES IN THE ICU (TABLE 23.4) +

Drug	Load	Maintenance	Onset / duration	Adverse effects
Propofol IV	—	5-50 mcg/kg/min (50-200 mcg/h); titrate every 5 min	1 min / 3-10 min	PRIS, vasodilation, negative inotropy, $\uparrow$ triglycerides
Midazolam IV	1-5 mg	1-5 mg/h	<5 min / 30 min	$\uparrow$ delirium risk
Lorazepam IV	1-4 mg	1-5 mg/h	15 min / 6-8 h	$\uparrow$ delirium risk

PEARLS

- Record the score and the stimulus used; reassess after every dose change
- Positive = agitation (look for pain, hypoxia, delirium, withdrawal); negative = sedation
- RASS -4/-5: GCS and airway first — not a target outside ventilated care